

This handout is for educational reference only. It is not medical advice. Always follow your prescription label, your pharmacy-specific instructions, and your KORB provider's guidance. If you have questions, contact KORB using the details at the end of this handout.

What Sermorelin is

Sermorelin is a synthetic peptide that stimulates your pituitary gland to release growth hormone (GH) naturally. It belongs to a class of compounds called growth hormone releasing hormone (GHRH) analogs.

The key distinction: Sermorelin does not replace growth hormone. It encourages your own pituitary gland to produce and release GH through its normal pathways. Because it works with your body's existing system, it is considered a more physiologic approach than direct GH replacement.

Growth hormone release triggered by Sermorelin also stimulates your liver to produce IGF-1 (insulin-like growth factor 1). Your IGF-1 level is one of the key markers your provider will monitor throughout your program.

What it may support

This medication is one possible adjunctive support option for patients with goals related to:

- Sleep quality, particularly deep, restorative sleep
- Recovery, both exercise recovery and general recovery physiology
- Body composition, lean mass preservation and fat metabolism as part of a lifestyle program
- Energy and wellness, a general sense of vitality
- GH-axis support as part of a structured longevity program

Response to Sermorelin varies from patient to patient. Some patients notice changes within the first few weeks; others notice little or nothing over an entire cycle. This is normal. Sermorelin is an adjunctive support option, not a cure, not a guaranteed treatment, and not a replacement for lifestyle foundations.

How to use it

Your prescription label, or the instructions given to you by your KORB clinical provider, is the authoritative source for your dose and directions. Some pharmacies label vials "as directed by provider" rather than printing exact instructions. What follows is a general reference. Do not adjust anything without speaking to your provider first.

How to inject	Subcutaneous (SQ) injection — fatty tissue under the skin
When to inject	Bedtime · Empty stomach (2+ hrs after eating)

Schedule	6 days on, 1 day off
Active weeks	Weeks 1–12 of your 16-week cycle
Off weeks	Weeks 13–16, a 4-week washout before your next cycle

Timing matters

Sermorelin should be injected at bedtime, on an empty stomach. GH is released naturally in pulses during sleep, particularly during the first few hours of deep sleep, and injecting at bedtime aligns with that rhythm. Food, particularly carbohydrates, can blunt GH release. Wait at least two hours after your last meal before injecting.

The rest day is intentional

Do not inject on your rest day, and do not make up a missed dose on it. If you miss a dose on a scheduled day, skip it rather than doubling up the next day.

Storage and handling

What to do	Detail
Refrigerate	36°F – 46°F (2°C – 8°C)
Do not freeze	Freezing damages medication
Protect from light	Keep away from direct sunlight
Discard at 28 days	Write the open date on the vial

Before first use: refrigerate, protect from light, do not freeze or shake.

After opening: write the open date on your vial and discard 28 days after first use, even if medication remains. If your label shows an earlier beyond-use date, follow the earlier one.

Do not use if the solution appears cloudy or discolored, or contains particles.

Traveling with your medication

Traveling does not have to interrupt your program. Keep your medication with you, in your carry-on or personal item rather than checked baggage, in its original labeled vial or packaging. It does not need to be packed in ice during travel; keep it away from excessive heat (above 86°F / 30°C) and from freezing, and refrigerate it again when you arrive. Medication and syringes are permitted in carry-on luggage, and TSA recommends keeping the original pharmacy label visible. Crossing time zones can shift your injection time by a few hours without a problem; if you will be off your usual schedule for several days, contact KORB.

What to expect

Timeline	Phase	What to expect
Weeks 1–4	Starting out	Some patients notice early changes in sleep quality or morning energy.
Weeks 4–8	Early response	Many notice nothing yet. This is normal.
Weeks 8–12	Continued support	Sleep quality, recovery and energy may begin to improve. Do not adjust your schedule.
Weeks 13–16	Off cycle — labs and lifestyle	No injections; this is intentional. Your labs, drawn between weeks 12 and 14, are reviewed during this window, and it is a good stretch to focus on nutrition, exercise and sleep before your next cycle. Do not restart early without provider direction.

Do not compare your response to others. Response is highly individual. Age, baseline GH levels, body composition, sleep quality and lifestyle habits all influence it.

Side effects and what to watch for

What you may notice

What you may notice	What to do
Injection site redness or irritation	Common, usually mild. Rotate injection sites. Tell your KORB provider if it persists.
Flushing or warmth after injection	Typically brief, and usually settles within minutes.
Headache	May occur early in therapy. Usually mild and short-lived.
Fatigue	Uncommon. Tell your KORB provider if it persists.

Tell your KORB provider at your next visit

What you may notice	What to do
Water retention or puffiness	Can occur with GH stimulation. Tell your KORB provider if noticeable.
Joint discomfort or tingling	May indicate a dose adjustment is needed. Tell your KORB provider.
Sleep changes	Some patients report improved sleep; rarely, disrupted sleep early on.

Stop injecting and seek emergency care immediately if you have trouble breathing, swelling of the face, lips, tongue or throat, a severe rash or hives, chest pain, or any symptom that feels severe or unsafe.

Lab monitoring

Lab monitoring is a required part of your program, not optional. Your provider uses labs to assess your response and your safety.

- IGF-1, your primary response marker. The target is the physiologic range; higher is not better.
- HbA1c and fasting glucose, because GH stimulation can affect glucose metabolism.
- Thyroid (TSH, free T4, free T3). Sermorelin requires a functioning thyroid axis.
- CBC, CMP, lipid panel, copper, zinc and ceruloplasmin, at baseline and every 16-week follow-up.
- PSA, added for men aged 45 and older at every draw.

Your lab draw should be completed between weeks 12 and 14 of your active cycle, so results are back in time for your 16-week follow-up visit. Some results, such as IGF-1, can take about a week. Unless your lab order says otherwise, plan to fast overnight and have blood drawn first thing in the morning.

Safety reminders

- Use Sermorelin only as prescribed. Do not change your dose, schedule or route without provider direction.
- Do not combine it with other GH-axis therapies unless KORB specifically instructs you to.
- Sermorelin is not appropriate during active malignancy, uncontrolled thyroid disease, or pregnancy.
- Do not restart after your off weeks early. The washout period is intentional.
- Tell your KORB provider if you develop a new medical condition, start a new medication, or become pregnant.
- Use a new insulin needle and syringe for every injection. Do not reuse.
- Never mix peptides. Use a separate needle and syringe for each peptide and each injection; do not combine them into one syringe.
- Follow your state or local rules for sharps disposal. Use an FDA-cleared sharps container, and do not put used needles or syringes in household trash unless your local guidance specifically allows it.

When to contact KORB

Contact KORB Operations	Patient Portal — message your provider	Seek emergency care immediately
- Questions about timing, storage or administration - Scheduling, billing and shipping Email: info@korbhealth.com Phone: +1 (888) 959-7299 Hours: Mon–Fri, 9 AM–6 PM CT	- Injection-site irritation or redness that does not resolve - Mild but persistent side effects - Questions about whether to continue therapy	- Trouble breathing, or swelling of the face, lips, tongue or throat - Severe rash or widespread hives - Chest pain or fainting - Severe or rapidly worsening symptoms - Confusion or severe weakness - Any symptom that feels urgent or unsafe

The Patient Portal is the only HIPAA-compliant way to message your provider directly. Providers check portal messages once per day on their clinic days. For anything that does not need your provider's direct medical judgment, contact KORB Operations by phone or email instead.

Phone and email are not appropriate for emergencies. When in doubt, go to urgent care or the emergency room.

Key reminders

- Sermorelin stimulates your own GH. It is not GH replacement.
- Write the open date on your vial and discard 28 days after first use.
- Your washout period is intentional. Do not restart early.
- Labs between weeks 12 and 14 are required before your 16-week follow-up visit.
- Response varies, and more is not better. Do not adjust your dose without provider direction.
- Contact KORB Operations for non-urgent questions, your provider through the Patient Portal for medical questions, and emergency care for severe symptoms.